

HOW TO OPEN A BEHAVIORAL HEALTH RESIDENTIAL FACILITY in Arizona

A licensed 24-hour residential treatment setting (BHRF) — ADHS-licensed, AHCCCS-funded.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate, code & fee should be re-verified before you rely on it.

The Arizona BHRF model — read this first

A Behavioral Health Residential Facility (BHRF) is a licensed, 24-hour residential treatment setting for adults (and, in some subclasses, youth) whose mental-health or substance-use condition is serious enough to need round-the-clock supervision and support while they are in treatment — but who do not need a hospital. It is NOT a sober living home (that's housing without treatment, licensed separately) and NOT an outpatient clinic. A BHRF is a health care institution licensed by the Arizona Department of Health Services (ADHS) under Article 7, and it is paid by AHCCCS (Arizona Medicaid) as a behavioral-health level of care. Opening one means clearing two big gates — the ADHS license and the AHCCCS contract — and planning for the one financial fact that surprises every new operator: the AHCCCS per-diem does NOT include room and board.

***You are opening a health care institution, not just a house.** A BHRF is a clinical, regulated, audited business — much closer to running a small treatment program than a group home. You are responsible for people in acute need, around the clock, and both ADHS and AHCCCS will hold you to it. The operators who succeed come in with clinical seriousness, enough capital to survive the months before revenue starts, and a real plan for the room-and-board gap. The ones who struggle treat it like a real-estate play and learn too late that the license, the staffing, and the documentation ARE the business.*

***The two gates, in one line.** (1) The ADHS license under Article 7 — without it you cannot legally operate. (2) The AHCCCS provider agreement and health-plan contracts — without them you have a licensed building but no way to get paid, because nearly every BHRF resident is an AHCCCS member. Run both tracks in parallel.*

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened a treatment program — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; BHRF rules are detailed and are actively updated by ADHS and AHCCCS, so verify each step with the agency named (see Part 12) before you rely on it. Where this guide gives a code, a fee, or a figure, treat it as a starting point to confirm — not a promise.

The journey at a glance

Ten milestones take you from idea to an operating, AHCCCS-contracted BHRF.

1	Decide your model	Population (mental health, substance use, or both), capacity, subclass (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance — the business foundation (Part 3)
3	Secure the property	A building that can pass residential physical-plant & fire at your capacity (Part 5)
4	Write your policies	The BHRF policy manual, forms & clinical documentation the license requires (Part 7)

5	Build your clinical team	Administrator, a Behavioral Health Professional, techs — all fingerprint-cleared (Part 6)
6	Get licensed by ADHS	The Article 7 application, fire inspection & licensing survey (Part 4)
7	Enroll with AHCCCS	Provider Type B8 via APEP, then contract with the ACC health plans (Part 9)
8	Solve room & board	Plan how you cover the housing the per-diem does not pay (Part 9)
9	Build your referral pipeline	Health-plan care managers, crisis, hospitals, courts, outpatient (Part 11)
10	Open & take referrals	Admit on medical necessity & authorization; treat, document, discharge (Part 8)

Rule of thumb. *The license is the long pole, and fire/life-safety is where residential applications stall — so handle the building and the fire marshal first, not last. Start your AHCCCS enrollment and plan contracting in parallel with licensing so your contracts are close to ready when your license issues. And model the room-and-board gap before you sign a lease.*

PART 1

What This Business Is

In short: A licensed 24-hour residential behavioral-health treatment program. Residents live at the facility while they participate in structured treatment, with the goal of stabilizing and stepping down to a lower level of care. ADHS-licensed, AHCCCS-funded.

A BHRF provides 24-hour treatment, supervision, and support to people whose mental-health or substance-use condition is serious enough that they cannot yet live safely on their own, but who do not need inpatient hospital care. Residents live at the facility while they participate in structured treatment — counseling, skills building, symptom and medication-management support, substance-use treatment, and daily structure — with the goal of stabilizing and stepping down to a lower level of care in the community. In Arizona's system a BHRF is a defined level of care that a member is placed into by their treatment team when they meet medical-necessity criteria — not a service a person simply walks in and buys.

What a BHRF is NOT

- **Not a sober living home.** A sober living home provides substance-free housing and peer support but NO treatment, and is licensed separately under a different Arizona law. A BHRF provides actual behavioral-health treatment and is licensed as a health care institution.
- **Not an outpatient clinic.** An Outpatient Treatment Center (licensed under Article 10) delivers behavioral-health services without residents living there. A BHRF is 24-hour and residential.
- **Not a psychiatric hospital.** A BHRF is a non-hospital residential level of care — below inpatient, above outpatient.

Subclasses to know

Beyond the standard BHRF, Arizona licenses a Secured BHRF (locked, court-ordered placements under A.R.S. § 36-425.06), an Adult Behavioral Health Therapeutic Home (a smaller, family-style setting), and a recidivism-reduction subclass for adults leaving the justice system. Most new operators open a standard BHRF; the others carry extra requirements. Decide which one you are before you apply — it changes your rules, your building, and your staffing.

Substance use, mental health, or both

A BHRF can serve members with mental-health conditions, substance-use conditions, or both, and both Arizona's rules and AHCCCS's medical-necessity criteria cover the full range. Your choice shapes your clinical model: a substance-use-focused BHRF works within the ASAM levels of care and lives with Certification-of-Need and Medication-Assisted-Treatment (MAT) requirements; a mental-health-focused BHRF centers on psychiatric stabilization, symptom management, and skills. Many facilities serve co-occurring conditions, which is common and often expected. Decide your population early, because it drives your staffing, your Behavioral Health Professional's expertise, your admission criteria, and the plans you contract with.

Key terms, in plain English. BHRF = Behavioral Health Residential Facility (your license). ADHS = AZ Dept. of Health Services (licenses you). AHCCCS = Arizona's Medicaid agency (pays you). Article 7 = the ADHS BHRF licensing rules (A.A.C. Title 9, Ch. 10, Art. 7). Provider Type B8 = the AHCCCS provider type for a BHRF. BHP = Behavioral Health Professional (your licensed clinical overseer). BHT = Behavioral Health Technician (your direct-care staff). Per-diem (H0018) = the bundled daily AHCCCS treatment rate — which excludes room & board. ACC plan = an AHCCCS Complete Care managed-care plan you contract with. FCC = Fingerprint Clearance Card. ASAM = the level-of-care framework for substance-use care. MAT = Medication-Assisted Treatment. NPI = National Provider Identifier.

Why this business — and why now

Arizona has leaned hard into community behavioral health. The state runs a robust crisis system, AHCCCS funds a full continuum of behavioral-health levels of care, and residential capacity is in genuine demand — for adults stepping down from psychiatric hospitalization, for people stabilizing from a substance-use crisis, and for members the courts and crisis system need to place quickly. Because AHCCCS pays for this level of care and members reach it

through an organized referral system, a well-run BHRF that keeps its beds authorized and its documentation clean can stay full. The need is real and, in most of the state, under-served.

Be honest with yourself about what you're taking on. A BHRF is a clinical, regulated, audited business, and the operators who fail usually fail for three business reasons, not clinical ones: (1) undercapitalization — they run out of cash covering payroll, rent, and food in the months before authorizations and payments stabilize; (2) weak clinical staffing — they cannot reliably staff a Behavioral Health Professional's oversight and 24-hour technician coverage, so quality and survey readiness slip; and (3) the room-and-board squeeze — they modeled the per-diem as if it covered housing. Solve capital, staffing, and the room-and-board plan up front, and the demand takes care of the rest.

PART 2

Who Regulates You — the Two Gates

In short: Two approvals, and you need both: the ADHS license (so you can legally operate) and the AHCCCS provider agreement plus health-plan contracts (so you can get paid). Plus the DPS Fingerprint Clearance Card program for your staff.

ADHS — Bureau of Residential Facilities Licensing (your licensor)

The Arizona Department of Health Services licenses your BHRF as a residential health care institution under Article 7. ADHS reviews your application, inspects the building, and runs the licensing survey that decides whether you open — and it inspects you again at relicensure. Without the ADHS license you cannot legally operate. (azdhs.gov)

AHCCCS and the AHCCCS Complete Care (ACC) health plans (your payer)

AHCCCS is Arizona's Medicaid agency, and nearly every BHRF resident is an AHCCCS member. You enroll with AHCCCS as a provider (Provider Type B8) through the AHCCCS Provider Enrollment Portal (APEP), then contract with the AHCCCS Complete Care (ACC) health plans that most members are enrolled in. AHCCCS sets the level-of-care and authorization rules and publishes the fee schedules; the plans do the contracting, credentialing, and day-to-day authorizations. AHCCCS Provider Services: 602-417-7670.

Arizona DPS — Fingerprint Clearance Cards

The Arizona Department of Public Safety issues the Fingerprint Clearance Card (Level One) that your administrator, clinical staff, and behavioral-health technicians must hold. Build fingerprinting into your hiring from day one — a missing or expired card is a common survey finding. (azdps.gov)

Adult Protective Services & the Department of Child Safety

You are a mandated reporter. Suspected abuse, neglect, or exploitation of an adult goes to Arizona Adult Protective Services (1-877-767-2385); concerns involving a minor go to the Department of Child Safety (1-888-767-2445); emergencies to 911. Build reporting into your policies and staff training.

What this means for you: the license lets you operate and the AHCCCS contract lets you get paid — and they run on separate clocks. The single biggest sequencing mistake new operators make is finishing the license and only THEN starting AHCCCS enrollment and plan contracting, which can add months of no revenue. Start both tracks together (Part 9), and keep a simple contact sheet from Part 12 on your office wall.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Arizona you form the LLC through the Arizona Corporation Commission (ACC); most operators actually form a corporation or LLC and many treatment programs choose an LLC taxed as an S-corp — ask your accountant. Note there is NO annual report for Arizona LLCs, a genuine advantage. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

1. **File your Articles of Organization with the ACC.** Online at ecorp.azcc.gov (Corporations Division, 602-542-3026). The filing fee is about \$50 (roughly \$85 expedited). Name the LLC and list a statutory agent (this can be you).
2. **Handle newspaper publication.** New AZ LLCs must publish a notice of formation in an approved newspaper — EXCEPT in Maricopa and Pima counties, where the ACC's own posting satisfies it. Check your county.
3. **Adopt an Operating Agreement.** The internal document that sets ownership and who can make decisions — your bank, your partners, and your licensing file will all want it.
4. **Get your local business license & TPT if required.** Most Arizona cities require a local business license; register for Transaction Privilege Tax (TPT) if your city requires it for your office.

Two Arizona quirks. Publication is required outside Maricopa/Pima counties, and there's NO annual report for AZ LLCs. Also line up your local city business license.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

1. **Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
2. **Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
3. **Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
4. **Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with AHCCCS as Provider Type B8 and bill the H0018 per-diem to the ACC plans. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

1. **Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
2. **Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

- 3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the behavioral-health residential taxonomy that fits a BHRF (for example, a Residential Treatment Facility, Mental Illness or Substance Abuse taxonomy) — confirm the exact code AHCCCS wants for Provider Type B8. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.
- 4. Submit — it's free and quick.** It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Arizona branches — Chase, Wells Fargo, Bank of America, or a local option like Arizona Financial or Desert Financial Credit Union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A BHRF houses vulnerable people in acute need around the clock, so you need coverage built for a residential treatment setting — and your AHCCCS plan contracts, your landlord, and ADHS will all want proof of it:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage at the facility, plus claims about the CARE and clinical judgment	Yes — foundational
Abuse & Molestation	Allegations of abuse of a resident by a staff member — general liability EXCLUDES this	Yes — very high exposure
Directors & Officers / Management Liability	Regulatory, employment, and governance claims against you and your leadership	Strongly recommended
Property & Business Interruption	Fire, damage, and lost income at the residential building	Yes
Non-Owned & Hired Auto	Staff driving their own cars, or transporting residents to appointments	Yes — critical
Workers' Compensation	Staff injuries (a 24-hour residential setting)	Required in AZ for employees

Get the abuse & molestation coverage explicitly. In a residential setting with vulnerable people, abuse & molestation is your highest-exposure line and is EXCLUDED by standard general liability — confirm it's included with a real limit, not a token sub-limit.

Real places to get it

Buy through an independent agent or broker who writes BEHAVIORAL-HEALTH and social-services programs specifically — this is a specialty niche, not a standard small-business policy. Carriers and programs active in behavioral-health residential include Philadelphia Insurance (PHLY), CNA, The Hartford, Great American, Markel, NSM/Irwin Siegel Agency (a social-services specialist), and Negley Associates. To find a local Arizona agent, use [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for a behavioral-health / substance-use residential program. Confirm the carrier writes BHRFs in Arizona before you rely on a quote.

What to ask the insurance agent

“Do you write behavioral-health residential facilities (BHRFs) in Arizona — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Do you include professional liability for the clinical care, plus non-owned & hired auto for transporting residents?”

“What limits do the AHCCCS Complete Care plans and my landlord require?”

“Can you set up Arizona workers' comp for a 24-hour residential staff?”

PART 4

Get Licensed by ADHS — Article 7, Step by Step

In short: *The ADHS license under Article 7 is the gate that lets you legally operate. You decide your capacity, build your policies and team, pass a fire inspection, and pass the ADHS licensing survey.*

4.1 Decide your capacity (and mind the 10-bed line)

Your licensed capacity — how many resident beds — drives almost everything: your building requirements, your staffing, and your rules. One threshold matters most for new operators: at 10 or more beds, a BHRF must have a clinical director, and the requirements get heavier. Many first-time operators deliberately license UNDER 10 beds (commonly 6–8) to keep the model manageable, prove they can run it, and scale into a larger facility later. Decide your capacity before you shop for a building, because it sets the physical-plant and fire requirements you must meet.

4.2 Build what the application requires (before you apply)

- ☐ Your formed entity, EIN, and a property that can meet residential physical-plant & fire at your capacity
- ☐ Your BHRF policy and procedure manual, forms, and clinical documentation (Part 7)
- ☐ Your administrator and Behavioral Health Professional identified (and clinical director if 10+ beds)
- ☐ Fingerprint Clearance Cards started for staff
- ☐ Architect-signed drawings if you build or remodel; a passed local fire inspection
- ☐ Your completed ADHS Article 7 initial application and fee

4.3 Apply, then pass the survey

- 1. Submit the ADHS Article 7 initial application.** File the Behavioral Health Residential Facility initial license application with ADHS (Bureau of Residential Facilities Licensing) with the fee, your drawings, and your fire inspection. Confirm the current application and license fees directly with ADHS before you rely on a number.
- 2. Pass the fire and physical-plant review.** ADHS and the local fire authority confirm the building meets the residential codes for your capacity — exits, egress, sprinklers/alarms as required, and occupancy (see Part 5).
- 3. Pass the ADHS licensing survey.** ADHS inspects the facility and reviews your policies, staffing, and documentation systems before issuing the license. Come to survey with your policy manual adopted, your team in place, and your clinical documentation forms ready to demonstrate.
- 4. Receive your license and keep it current.** Once licensed you may operate — and you'll be surveyed again at relicensure. Report changes as the rules require and keep every staff qualification and Fingerprint Clearance Card current.

What this means for you: ADHS is not looking for a perfect building alone — it's looking for a program that can safely treat people and prove it on paper. The applicants who clear the survey the first time are the ones who show up with the policy manual, the clinical team, the documentation forms, and the fire clearance already done. Treat the license as the long pole of your whole timeline, and start it early.

PART 5

The Building — Physical Plant, Fire & Life Safety

In short: *Because residents live at a BHRF around the clock, the physical building carries real weight in licensing — and fire/life-safety is the single most common reason a residential application stalls. Handle it first, not last.*

5.1 What the building must do

The facility must meet the residential physical-plant standards for your capacity — adequate bedrooms and bathrooms, living and treatment space, and the safety features the codes require — and pass a fire inspection from the local fire department. If you build or remodel, your drawings must be architect-signed and meet the adopted building and fire codes. Choose (or design) the building around your licensed capacity, not the other way around.

5.2 Fire & life safety — where applications stall

- 1. Involve the fire marshal early.** The local fire department's inspection — sprinklers or alarms as required, exits, egress, and occupancy for your capacity — is one of the most common reasons a residential license is delayed. Bring the fire marshal in at the very start, before you commit to a building.
- 2. Use an architect who knows health-care-institution codes.** If you remodel, an architect familiar with residential health-care-institution codes will save you months and rework. Health-care occupancy is not the same as an ordinary house.
- 3. Build extreme-heat safety in.** Because you house vulnerable residents in Arizona, build heat safety into the building and your emergency plan: reliable cooling, backup planning, and heat-illness awareness are not optional here.

Lease with a licensing contingency. *Before you sign a lease or purchase, have the fire marshal and an architect confirm the building can realistically reach occupancy at your capacity — and make your lease contingent on licensure where you can. Committing to a building that can't pass is the most expensive mistake in this whole process.*

PART 6

Build Your Clinical Team

In short: A BHRF runs on people: an administrator, a Behavioral Health Professional for clinical oversight, a clinical director at 10+ beds, and behavioral-health technicians providing 24-hour coverage — all fingerprint-cleared. Real BHP oversight is what passes both survey and audit.

6.1 The roles a BHRF must have

- **Administrator:** runs the facility day to day and is accountable to ADHS for compliance, staffing, and operations.
- **Behavioral Health Professional (BHP):** the licensed clinician (e.g., an independently licensed counselor, social worker, psychologist, or equivalent) who provides clinical oversight, completes or supervises assessments, and signs off on treatment plans. This is the clinical backbone of the license.
- **Clinical Director (at 10+ beds):** crossing the 10-bed line adds a required clinical director and heavier requirements — a reason many operators start under 10 beds.
- **Behavioral Health Technicians (BHTs):** the direct-care staff who provide the 24-hour supervision, structure, and support — scheduled to cover every hour, every day.

Nail down your BHP relationship — employed or contracted — before you open, and price their time into your model. A BHRF that cannot demonstrate real BHP oversight will fail both survey and audit; the BHP is not a box to check but the clinical engine of the program.

6.2 Fingerprint Clearance Cards and qualifications

Every staff member who works with residents needs a Level One Fingerprint Clearance Card from Arizona DPS, plus the qualifications their role requires. Keep each person's card, license, training, and competency records on file and current — your surveyor and any AHCCCS audit will ask for them. Start fingerprinting the moment you make an offer; the clearance can take time and no one should work with residents without it.

6.3 Recruiting and keeping clinical staff — the real make-or-break

After licensing, your hardest ongoing job is keeping a Behavioral Health Professional engaged and your technician shifts fully covered. Staffing is where quality, survey readiness, and census all live or die.

- **Recruit continuously:** post for BHTs constantly (Indeed, local groups, referrals with a bonus), and build a bench so a call-out never leaves a shift uncovered. A 24-hour residential setting cannot run short.
- **Secure your BHP early and formally:** a written employment or contract agreement, clear hours, and enough of their time to actually oversee the caseload — not a name on a form.
- **Onboard to the documentation standard:** train every tech on the assessment, treatment-plan, and progress-note expectations from day one, because their documentation is what gets the per-diem paid.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real supervision and support, and recognition. Turnover in a 24-hour program is expensive and destabilizing for residents — invest in keeping good staff.

PART 7

Policies, Documentation & Getting Operational

In short: Your policy manual and your clinical documentation are not paperwork — they are the license and the revenue. AHCCCS pays for what your assessment, treatment plan, BHP sign-off, and continued-stay justification document.

7.1 Your BHRF policy and procedure manual

Adopt a policy and procedure manual to the Article 7 standard before you apply: resident rights and grievances, admission and discharge criteria, assessment and treatment planning, medication support and MAT, supervision and staffing, incident and abuse/neglect reporting, emergency and disaster planning (including extreme heat), infection control, and quality management. This manual is both what ADHS reviews at survey and your operating backbone once you open.

7.2 Clinical documentation — what AHCCCS actually pays for

In Arizona's system the money follows the documentation. Build these into your workflow from the first admission:

- **The behavioral-health assessment:** completed at admission (generally within 48 hours) and supporting medical necessity for the residential level of care.
- **The treatment plan:** developed with the member's outpatient treatment team, with BHP sign-off, driving the care and the daily documentation.
- **Continued-stay justification:** ongoing documentation that the member still meets criteria — this is what keeps the authorization (and the per-diem) alive.
- **Daily service and supervision notes:** the record that the treatment and 24-hour supervision in the plan actually happened — the backup for every billed day.

7.3 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 8

How Care Works — Admission to Discharge

In short: A BHRF is a level of care a member is placed into, not a service someone buys. The clinical flow IS the billing flow: referral and authorization, assessment and plan, treatment, continued-stay, and discharge to a lower level of care.

Stage 1 — Referral & authorization

A member is referred by their outpatient or inpatient treatment team, crisis services, a hospital, or the courts. Admission requires that the member meet medical-necessity criteria and — for AHCCCS members — PRIOR authorization, with continued authorization to keep the stay going.

What you confirm before you admit:

- The member's plan and eligibility; the prior authorization for the BHRF level of care; for substance-use residential, the Certification of Need with an ASAM assessment supporting the level; and that the placement fits your population, capacity, and scope.

Stage 2 — Assessment & treatment plan

A behavioral-health assessment is completed at admission (generally within 48 hours), and a treatment plan is developed with the outpatient treatment team, with BHP sign-off. The plan drives the care and the documentation.

Stage 3 — The treatment itself

The BHRF provides the services in the plan — counseling and therapy support, skills building, symptom and medication-management support, substance-use treatment, structure, and 24-hour supervision — as a bundled level of care.

You cannot exclude MAT members. A BHRF cannot turn away a member because they are on Medication-Assisted Treatment for opioid use disorder; Arizona law requires that MAT members be admitted and supported. Build MAT coordination into your model.

Stage 4 — Continued stay & reassessment

The team reassesses progress and risk, documents that the member still meets criteria, and keeps the authorization current. Delivered days without a current authorization and supporting documentation don't get paid.

Stage 5 — Discharge to a lower level of care

The goal of the whole stay is a stable step-down. The team plans discharge to outpatient or a lower level of care, coordinates with the outpatient team and health plan, and documents the transition. A good discharge is both the clinical goal and what earns you the next referral.

You are part of a team, not an island: a BHRF works with the member's outpatient treatment team, which is involved in the plan and the discharge. Services the member needs that your BHRF does not provide are arranged and authorized separately through that team — not added onto your per-diem.

PART 9

How You Get Paid — AHCCCS, the Per-Diem & Room and Board

In short: Nearly every BHRF resident is an AHCCCS member. You enroll as Provider Type B8, contract with the ACC plans, and bill a per-diem (H0018) for the treatment — which does NOT include room and board. Solving room and board is the defining financial challenge.

9.1 Enroll with AHCCCS, then contract with the plans

- 1. Enroll through APEP as Provider Type B8.** Register as an AHCCCS provider in the AHCCCS Provider Enrollment Portal as the behavioral-health residential provider type (B8). This makes you a recognized Medicaid provider — but does not by itself let you serve any given member.
- 2. Contract with the AHCCCS Complete Care (ACC) plans.** Most members are enrolled in an ACC managed-care plan; to serve a plan's members and be paid, you contract with that plan and join its network. Approach each ACC plan whose members you want to serve and complete its contracting and credentialing.
- 3. Understand the fee-for-service path.** Members who are not in a plan are authorized and paid directly through AHCCCS (fee-for-service). Enrolling with AHCCCS and contracting with a plan are two different steps — you generally need both.
- 4. Start early, in parallel with licensing.** Contracting and credentialing take time; begin as soon as you can so your contracts are close to ready when your license issues.

9.2 The per-diem — and the room-and-board catch

AHCCCS pays for a BHRF as a per-diem: one bundled daily rate that covers the treatment, supervision, and BHP oversight for a 24-hour day. The BHRF per-diem is billed with HCPCS code H0018 (short-term residential); a long-term residential per-diem, H0019, also exists. Because the per-diem is a bundle, you generally cannot separately bill the counseling, the BHP's oversight, or the nursing that happen inside the BHRF — those are built into the daily rate. The stay requires prior and continued authorization, and for substance-use residential expect a Certification of Need with an ASAM assessment attached.

Code	What it is	Notes
H0018	Behavioral-health short-term residential, per diem	The core BHRF per-diem; WITHOUT room and board; prior auth required
H0019	Behavioral-health long-term residential, per diem	Longer-stay residential per-diem
Room & board	Housing and meals — NOT in the per-diem	Covered by resident income/SSI, a room-and-board charge, or other funding — not Medicaid

Codes are national HCPCS references; AHCCCS and your health plan set the exact code, modifier, rate, and authorization rules. Claims are generally submitted on the CMS 1500 (837P) through the AHCCCS portal or the plan. Verify everything with AHCCCS and your contracted plan before you bill.

The single most important financial fact. The AHCCCS per-diem does NOT include room and board. The daily rate pays for TREATMENT — not the bed, the meals, or the roof. Medicaid generally cannot pay room and board in this setting, so you must cover it another way: the resident's own income or SSI, a modest disclosed room-and-board charge, grant or state/county funding, or serving some private-pay or commercially insured residents. Operators who model the per-diem as if it covered housing run out of money. Build your room-and-board plan before you open.

9.3 Finding your actual per-diem rate

This guide does not print a per-diem dollar figure on purpose — because there is no single number that applies to every BHRF, and the right figure for you is one you should confirm, not assume. AHCCCS publishes dated behavioral-health fee schedules on its Rates and Billing pages in two versions — a fee-for-service (FFS) schedule and a managed-care (MCO) schedule. Look up H0018 on the current-dated schedule for the published rate, but treat the FFS figure as a reference point: most residents are in ACC plans, and each plan sets the rate it actually

pays under your contract. To pin down your real number: look up H0018 on the current AHCCCS behavioral-health fee schedule, ask each ACC plan for its BHRF per-diem in writing, and email AHCCCS at FFSRates@azahcccs.gov with rate questions. Build your budget on the rate your contracted plans confirm in writing — not a figure from an article or a competitor.

9.4 The payer mix and solving the room-and-board gap

Source	What it covers	What to know
AHCCCS (ACC plans / FFS)	The BHRF treatment per-diem	Your primary payer; requires enrollment, a plan contract, and authorization
Resident income / SSI	Room and board	The most common way residents cover the housing the per-diem excludes
Private pay / commercial	Treatment and/or room and board	Some operators serve private-pay or commercially insured residents alongside AHCCCS
Grants / state & county	Room and board, gaps	Behavioral-health and housing funding streams can help cover non-Medicaid costs

Because the room-and-board gap is the defining financial challenge of a BHRF, it deserves a real plan, not a hope. The common approaches, often combined: collect the resident's own income or SSI toward room and board; set a modest, disclosed room-and-board charge within what residents can actually pay; pursue behavioral-health housing subsidies and state or county funding that exist precisely to cover the housing Medicaid will not; and, where it fits your model, serve some private-pay or commercially insured residents whose payment can include room and board. Model it conservatively — assume vacancy, assume some residents cannot pay much, and make sure the treatment per-diem plus your room-and-board plan actually covers rent, food, and staffing with margin.

PART 10

What It Costs & How Long It Takes

In short: Budget the building and fire/life-safety work (often the biggest cost), licensing, your clinical team's payroll before revenue starts, insurance, and several months of operating runway. The license and contracting drive your timeline.

10.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
Business setup	ACC LLC ~\$50 (+ publication if applicable), EIN free, NPI free, city license, legal	~\$50–\$200 + legal
Property & build-out	Lease or purchase + any remodel to meet residential physical-plant & fire	Your largest variable cost
Fire / life-safety	Sprinklers/alarms, egress, architect-signed drawings as required	Get local bids early
ADHS license	Article 7 initial application + license fee	Confirm current fees with ADHS
Insurance	Liability + abuse & molestation + property + auto + workers' comp	Get quotes — annual
Clinical team payroll	Administrator, BHP, techs — BEFORE the per-diem starts	Months of runway
Policies & systems	Policy manual, clinical documentation/EHR, billing setup	One-time + monthly SaaS
Operating runway	Rent, food, payroll before authorizations & payments stabilize	Several months — critical

10.2 The money math — how a BHRF actually pencils out

BHRF economics are different from an hourly-care business: your revenue is a per-diem times occupied, authorized beds, and your two biggest realities are occupancy and the room-and-board gap. Lay it out with your own numbers:

- **Revenue per day:** the confirmed per-diem (from your ACC plan contract) times the number of beds that are BOTH filled AND currently authorized — not just filled. An unauthorized or undocumented day is an unpaid day.
- **Room and board, added separately:** the resident income/SSI, room-and-board charge, or other funding that covers the housing the per-diem does not — modeled conservatively.
- **Against fixed costs:** rent, food, insurance, the BHP's time, and 24-hour technician payroll run whether a bed is full or empty — so your break-even is an OCCUPANCY number. Know how many authorized beds you must keep filled every day to cover fixed costs, and treat census and authorization discipline as the whole game.
- **What this means for you:** a BHRF that fills its beds but lets authorizations lapse or documentation slip bleeds cash even at full occupancy — and one that solves census and documentation but never solved room and board bleeds on rent and food. Model both before you sign a lease.

10.3 Timeline

Months 1–2	Form & secure the property	Entity, EIN, insurance; a building that can meet physical-plant & fire at your capacity
Months 1–3	Team & policies	Recruit the administrator & BHP (clinical director if 10+ beds); adopt the policy manual; start Fingerprint Clearance Cards
Months 2–4	Build & apply	Complete any remodel with architect-signed drawings; pass the fire inspection; submit the ADHS Article 7 application
Months 3–5	License & enroll	Pass the ADHS survey and receive your license; in parallel enroll with AHCCCS (B8) and contract with the ACC plans
Months 5–6	Open	Finalize the room-and-board plan; build referral relationships; accept your first

		authorized admissions
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PART 11

Getting Your First Residents

In short: A BHRF fills its beds through the behavioral-health system, not through family marketing. Your census is your reputation with health-plan care managers, crisis and hospital discharge planners, courts, and outpatient teams.

11.1 Where your residents come from

- **The AHCCCS Complete Care health plans:** their care managers place members into residential care with contracted, in-network BHRFs — being contracted and reliable is what gets you in the rotation.
- **Outpatient behavioral-health clinics:** a member's outpatient team refers up to residential when someone needs more than outpatient care, and stays involved through discharge.
- **Crisis services & psychiatric hospitals:** the crisis system and inpatient units need somewhere to step members down to; a BHRF that answers the phone and admits smoothly becomes their go-to.
- **Courts, probation & justice programs:** court-ordered and justice-involved placements are a steady source, especially for the secured and recidivism-reduction subclasses.

11.2 What to ask each referral source

“How do you decide which BHRF to place a member with, and what puts an agency on your preferred list?”

“What do you need from me to be in-network and in your placement rotation — contract, credentialing, insurance, capacity?”

“Who is the care manager, discharge planner, or liaison I should build a relationship with?”

“How fast do you need us to be able to admit after a referral, and what makes an admission easy for you?”

“What gets a facility dropped from your list?”

The through-line is reliability: referral sources place members with the facilities that pick up the phone, admit without drama, communicate, and discharge well. Your reputation with a handful of health-plan care managers and hospital discharge planners is, in practice, your census — so treat those relationships as core operations, not marketing.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact and verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the building & staff

Who	Contact	What to verify / ask
ADHS — Residential Facilities Licensing	azdhs.gov	What a complete Article 7 initial application requires for my capacity; current fees & timeframes
Local fire department / fire marshal	your city/county fire authority	What I need to pass a residential fire inspection at my capacity
Architect (health-care-institution codes)	a licensed AZ architect	Whether the building can reach occupancy at my capacity; drawings for remodel
AZ DPS — Fingerprint Clearance Cards	azdps.gov	How my staff apply for Level One cards

Business, insurance & billing IDs

Who	Contact	What to verify / ask
AZ Corporation Commission	602-542-3026 · ecorp.azcc.gov	Forming your LLC (~\$50); publication in your county; no annual report
IRS	irs.gov (EIN Assistant)	Getting your EIN (free)
NPPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI and the correct behavioral-health taxonomy
Insurance agent (behavioral-health program)	trustedchoice.com; carriers in Part 3.5	Liability + abuse & molestation + property + auto + workers' comp

Enrollment, payment & reporting

Who	Contact	What to verify / ask
AHCCCS — Provider Enrollment (APEP)	azahcccs.gov/APEP	Enrolling as Provider Type B8 (behavioral-health residential)
AHCCCS — Provider Services & rates	602-417-7670 · FFSRates@azahcccs.gov	H0018 on the current fee schedule; authorization & billing rules
AHCCCS Complete Care (ACC) plans	the plans serving your area	Contracting & credentialing; the BHRF per-diem they pay; authorization rules
Adult Protective Services / DCS	1-877-767-2385 (adults) · 1-888-767-2445 (minors)	Mandatory reporting of abuse, neglect, or exploitation

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to take AHCCCS?

In practice, yes — nearly every BHRF resident is an AHCCCS member, so the AHCCCS enrollment (Provider Type B8) and ACC plan contracts are how a BHRF gets paid. Some operators serve private-pay or commercially insured residents alongside AHCCCS, but a BHRF built to ignore Medicaid will struggle to fill beds. Start your enrollment and contracting in parallel with licensing.

What's the real difference between a BHRF and a sober living home?

A sober living home provides substance-free housing and peer support but NO treatment, and is licensed separately under a different Arizona law. A BHRF provides actual behavioral-health treatment, is licensed by ADHS as a health care institution under Article 7, and is paid by AHCCCS as a level of care. They are different licenses, different rules, and different money — don't confuse them.

Why does everyone warn about room and board?

Because the AHCCCS per-diem pays for treatment, not housing — and Medicaid generally cannot pay room and board in this setting. Operators who model the per-diem as if it covered the bed, the meals, and the roof run out of cash. You cover room and board separately (resident income/SSI, a modest charge, grants, or private pay), and you plan it before you open.

Should I start small or go big?

Most first-time operators deliberately license under 10 beds (commonly 6–8) to stay below the clinical-director threshold, prove they can run the program cleanly, and scale into a larger facility later. Crossing 10 beds adds a clinical director and heavier requirements — grow into it rather than starting there.

Two worked examples

Example 1 — A small mental-health BHRF

You license an 8-bed BHRF (under the 10-bed line, so no clinical director required) for adults with serious mental illness. You employ an administrator, contract with a licensed BHP for clinical oversight, and staff behavioral-health technicians around the clock, all fingerprint-cleared. You pass the ADHS survey, enroll with AHCCCS as Provider Type B8, and contract with two ACC plans. Referrals come from outpatient clinics and the crisis system; each admission arrives with an assessment, a treatment plan, and prior authorization. AHCCCS pays your H0018 per-diem for treatment; residents' SSI covers room and board. You reassess and justify continued stay, and discharge each resident to outpatient care.

Example 2 — A substance-use residential BHRF

You open a BHRF focused on substance-use treatment. Admissions arrive with a Certification of Need and an ASAM assessment supporting the residential level, and you cannot turn away members on Medication-Assisted Treatment — you build MAT coordination in. Your BHP oversees substance-use counseling and the treatment plan; the outpatient team stays involved and handles services outside your scope. You bill the H0018 per-diem with the required authorization, cover room and board through resident income and a modest charge, and measure success by stable step-downs to outpatient treatment. Your referral partners are hospitals, crisis facilities, courts, and outpatient SUD providers.

You can do this

A BHRF is a licensed treatment program, not a house with residents. Clear the two gates — the ADHS Article 7 license and the AHCCCS contract — build a real clinical team around a Behavioral Health Professional, plan for the room and board the per-diem does not pay, and run the facility on medical necessity, authorization, and documentation. Do those, and you have a business that fills a serious, under-served need and stays full. Take it one rung at a time, verify each fee and contact as you go, and lean on the people in Part 12.

Reminder. Prepared for PolicyReady.org and grounded in A.A.C. Title 9, Chapter 10, Article 7 (R9-10-701 et seq.), A.R.S. § 36-425.06, and current AHCCCS information as of 2026. This is general information, not legal, tax, or clinical advice. Requirements, fees, rates, codes, and contacts change — verify each with ADHS, AHCCCS, and qualified counsel before you rely on it. Confirm the current Article 7 application and license fees, the H0018 per-diem your plans actually pay, and your capacity's staffing rules before you rely on them.